

BIOSYNC CLINICAL LABORATORY REPORT

Patient: Nina Vasquez (DOB: 07/03/1998) | Date: January 22, 2026

Vital Signs & Biometric Readings

Heart Rate	94 BPM
Blood Pressure	118/74 mmHg
SpO2	91%
Temperature	100.6°F
Glucose	168 mg/dL
Weight	105 lbs
BMI	19.2
HRV	38 ms

Clinical Notes

Patient presents with increased cough productive of yellow-green sputum, worsening dyspnea on exertion, and decreased appetite for 12 days. Tobramycin allergy documented. Currently on Orkambi 200/125mg twice daily, Albuterol nebulizer (frequency increased), Creon 12,000 units with meals, Azithromycin 500mg 3x/week, Vitamin ADEK daily, Dornase alfa daily. Heart rate 94 BPM — elevated, consistent with pulmonary exacerbation and infection. SpO2 91% — significantly below CF baseline, supplemental oxygen considered. Temperature 100.6°F — low grade fever, pulmonary exacerbation with infectious component. Glucose 168 mg/dL — elevated, CFRD-related glucose dysregulation worsening with infection stress. Weight 105 lbs — down 3 lbs from December, nutritional depletion concerning. HRV 38 ms decreased. FEV1 estimated 52% predicted — decline from baseline 62%.

Provider Notes

Pulmonary exacerbation confirmed. IV antibiotics via port-a-cath: Ceftazidime and Colistin (Tobramycin allergy documented — avoided). Course: 14 days. Albuterol nebulization increased to every 4 hours. Chest physiotherapy intensified to 3x daily. High-calorie supplemental nutrition (Boost Plus) added. Endocrinology notified of glucose spike — insulin sliding scale initiated. Patient to be monitored closely. If SpO2 drops below 89%, admit immediately. Port-a-cath accessed for IV therapy — home IV nursing arranged. Follow-up in 1 week. Ordering Provider: Dr. Angela Holt, MD — Pulmonology/CF Center. Facility: BioSync Clinical Health Center